NCCP 00525 · VERSION 6

# Ribociclib + endocrine therapy · metastatic

The usual metastatic starting dose is ribociclib 600 mg once daily on Days 1–21 followed by Days 22–28 off. Continue the prescribed AI daily or fulvestrant on its own injection schedule; add ovarian suppression when indicated. Continue while beneficial and tolerated.

## Make treatment fit into daily life

Take ribociclib once daily with or without food; swallow whole. Avoid grapefruit. If a dose is missed or vomited, do not take an extra dose; take the next scheduled dose. Confirm any clinical hold and restart with the team. The endocrine partner follows a separate prescription.

## Before starting

Confirm the exact medicine, strength, endocrine partner, baseline tests and first review appointments. Ask who arranges tests and who reviews the results.

## Taking treatment

Take ribociclib once daily with or without food; swallow whole. Avoid grapefruit. If a dose is missed or vomited, do not take an extra dose; take the next scheduled dose. Confirm any clinical hold and restart with the team. The endocrine partner follows a separate prescription.

## Your endocrine partner has a separate timetable

An aromatase inhibitor is normally taken daily through the targeted-medicine rest week. If fulvestrant is the chosen partner, the usual schedule is 500 mg by intramuscular injection on Days 1, 15 and 29, then once monthly; confirm the prescribed dose and appointment dates. Any ovarian-suppression injections have their own schedule.

## Treatment duration

There is no fixed three-year stop in this metastatic regimen; the team reviews response, side effects and the next treatment decision.

## Supply and appointments

Request the next supply before tablets run out. Blood tests and any ECG or injections need their own appointments. This oral treatment does not itself require a central line or infusion pump.

## If treatment is held

Ask which medicine is held, whether the endocrine partner continues, what needs to recover, when the next review is due and who will authorise restart.

## Support between appointments

Keep the oncology contact details and prescribed symptom medicines accessible. New severe symptoms need prompt assessment rather than waiting for the next blood test.

## Who arranges each check?

Agree the due window, booking team, result reviewer and whom to contact if an appointment or result is missing. This reference page does not book or record a test.

## Sources and review

The NCCP regimen supplies the treatment rules. Separate guidelines supply the longer-term monitoring recommendations, with their qualifications retained.


- NCCP 00525 v6 · official regimen
- NICE NG101 · bone health and follow-up
- HSE · chemotherapy side effects and urgent advice
- ASCO · biomarkers in metastatic breast cancer
- ESMO metastatic breast cancer guideline · response assessment
- NOGG · section 7, aromatase inhibitors and fracture risk
- eviQ · PICC patient information
- eviQ · implanted port patient information
- Novartis · Kisqali product information, updated September 2026
- MONALEESA-2: ribociclib with letrozole · primary report
- EMA Kisqali current product information; checked 2026-10-25
- MONALEESA-2 final OS, Hortobagyi et al 2022

- MONALEESA-2 updated PFS, Hortobagyi et al 2018
- MONALEESA-3 updated overall-population OS, Slamon et al 2021
- MONALEESA-7 exploratory extended OS, Lu et al 2022

Source reconciled; independent clinical, pharmacy and patient/PPI review pending